Sampson Physical Therapy

585 Raymond Field
Pasadena, TX
Phone 346-555-1390

Itemized Medical Bill

Patient:	Roger Hoffman
Date of birth:	
Insurance claim:	C-1432
Date of injury:	11/26/2023
First visit:	12/03/2023
Last visit:	03/22/2024
Number of visits:	18

Services

Service	Units	Charge
99203 New patient evaluation	1	$300.03
98941 Spinal manipulation	5	$503.13
97110 Therapeutic exercise	6	$1,113.02
97124 Massage therapy	4	$408.14
72040 X-ray, cervical spine	1	$901.31
20552 Trigger point injection	1	$633.15
TOTAL		$3,858.78

Recent visits

Date	Service	Charge
02/19/2024	97110 Therapeutic exercise	$188.22
02/28/2024	97110 Therapeutic exercise	$163.00
03/02/2024	97110 Therapeutic exercise	$167.48
03/07/2024	98941 Spinal manipulation	$103.47
03/17/2024	97124 Massage therapy	$80.48
03/22/2024	97110 Therapeutic exercise	$192.00

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Treatment History and Summary

Patient:	Roger Hoffman
Treating provider:	David Dickerson, DC
Diagnosis:	knee injury

Treatment history by month

Month and services	Visits	Charges
December 2023: chiropractic adjustment, evaluation, massage therapy, physical therapy	5	$889.13
January 2024: chiropractic adjustment, imaging, massage therapy, pain management	5	$1,036.51
February 2024: massage therapy, physical therapy	4	$639.71
March 2024: chiropractic adjustment, massage therapy, physical therapy	4	$543.43

Treatment summary

Patient presented following a low-speed head-on motor vehicle collision with right knee pain and swelling. The reported mechanism of injury is consistent with the kind and force of the impact observed. Initial examination revealed effusion and tenderness along the medial joint line, with positive findings on the valgus stress test, while other ligamentous and neurovascular assessments returned within normal limits. Imaging was reviewed and did not show acute fracture. No prior injury to the right knee is documented in the available history. Care was delivered over 18 visits consisting of physical therapy, anti-inflammatory medication, and activity modification. Treatment followed a typical progression, with visit frequency tapering as range of motion and pain improved. Billed services reflect the planned course of care, with no duplicate or repeated charges identified. At last follow-up the patient was improving and was discharged to a home exercise program with instructions to return if symptoms worsen.